

EBOLA VIRUS DISEASE

Democratic Republic of the Congo

External Situation Report 03

Date of issue: 30 September 2025

Data as reported by: 28 September 2025

Situation update	Cases	Deaths	CFR
	 64	 42	65.6%

The Ebola virus disease outbreak in the Democratic Republic of the Congo continues, with signs of a notable decline in transmission. Since our last update ([Situation Report #2](#)), a total of seven new cases have been reported, comprising six confirmed and one probable cases (retrospectively validated). The new cases were detected across three health areas within Bulape Health Zone, namely, Bulape ($n=4$), Mpianga ($n=2$), and Dikolo ($n=1$). During the same reporting period, seven deaths occurred among newly identified and previously hospitalized cases. The reported deaths were distributed across Dikolo ($n=3$), Bulape ($n=2$), Mpianga ($n=1$), and Bulape Communautaire ($n=1$) health areas.

As of 28 September 2025, a total of 64 cases (53 confirmed and 11 probable), including 42 deaths (31 confirmed, 11 probable), have been reported from Bulape Health Zone, Kasai Province, Democratic Republic of the Congo. The overall case fatality ratio (CFR) is 65.6%. Cumulatively, five confirmed cases have been reported among healthcare workers, including three deaths. The outbreak remains confined to six affected health areas out of the 21 that make up Bulape Health Zone.

Cases to date range in age from 0 (newborn) to 65 years. Since the onset of the outbreak, the majority of cases have been reported among females (57.8%, $n=37$), children aged 0–9 years (25.0%, $n=16$), and individuals aged 20–29 years (23.4%, $n=15$). Mortality has also been concentrated among these groups, with females accounting for over half of the reported deaths (57.1%, $n=24$) and children under 10 years representing 31.0% of deaths ($n=13$). The CFR is slightly higher among males (66.7%) compared to females (64.9%). In the past two weeks, fewer cases ($n = 2$) have been reported among children (0–9 years old). A decreasing CFR trend has been observed over time with improvement in surveillance (early case detection and isolation) and the quality of case management (prompt and high-quality treatment).

As of 28 September 2025, a total of 1 787 contacts were under follow-up, with 1 735 (97.1%) seen in the last 24 hours. Nine (9) cases were successfully treated and discharged following recovery while 13 are currently in treatment as of 28 September 2025.

Figure 1: Epidemic curve of confirmed and probable Ebola virus disease (EVD) cases in Bulape Health Zone, Kasai province, Democratic Republic of the Congo, 16 August – 28 September 2025

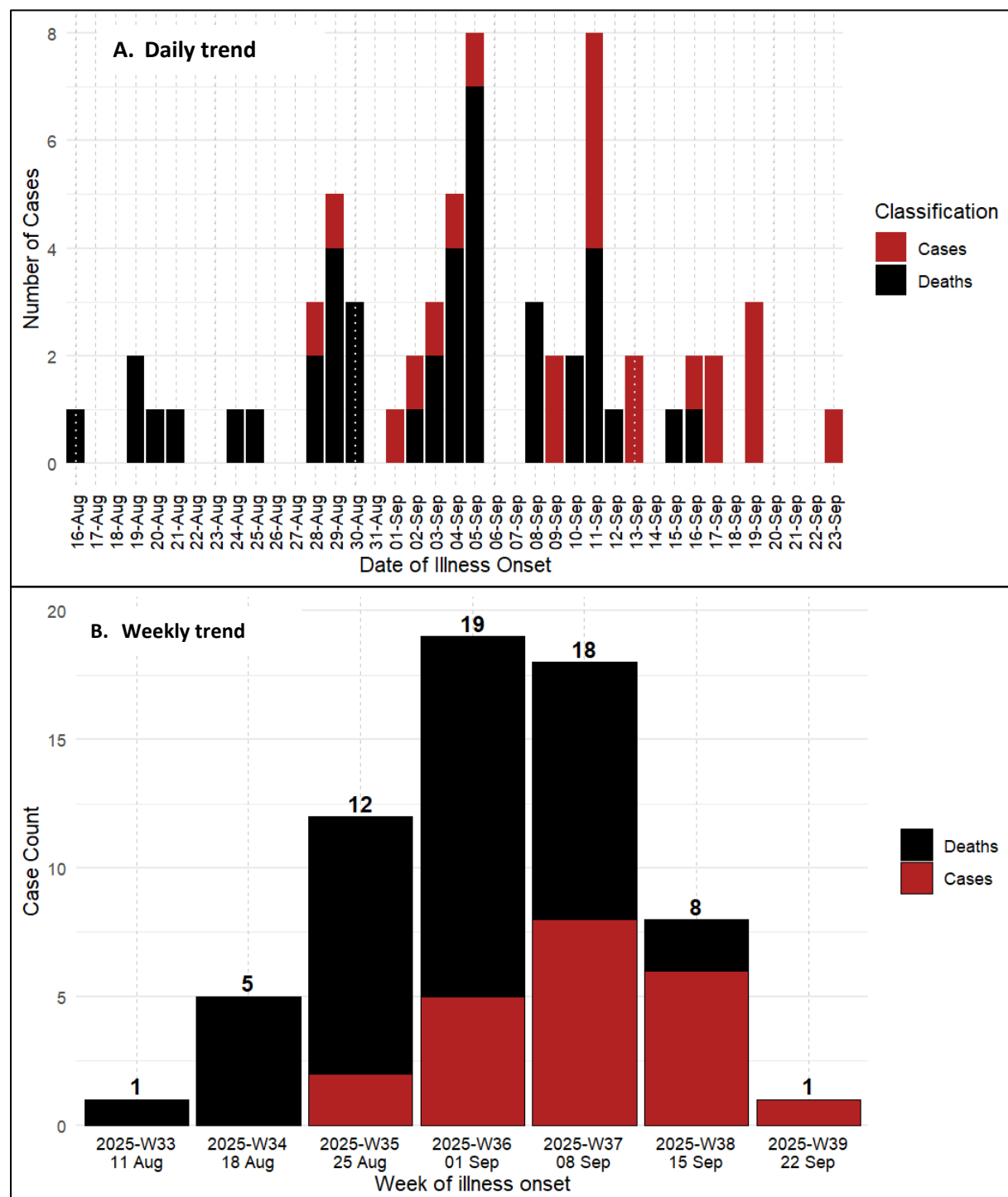
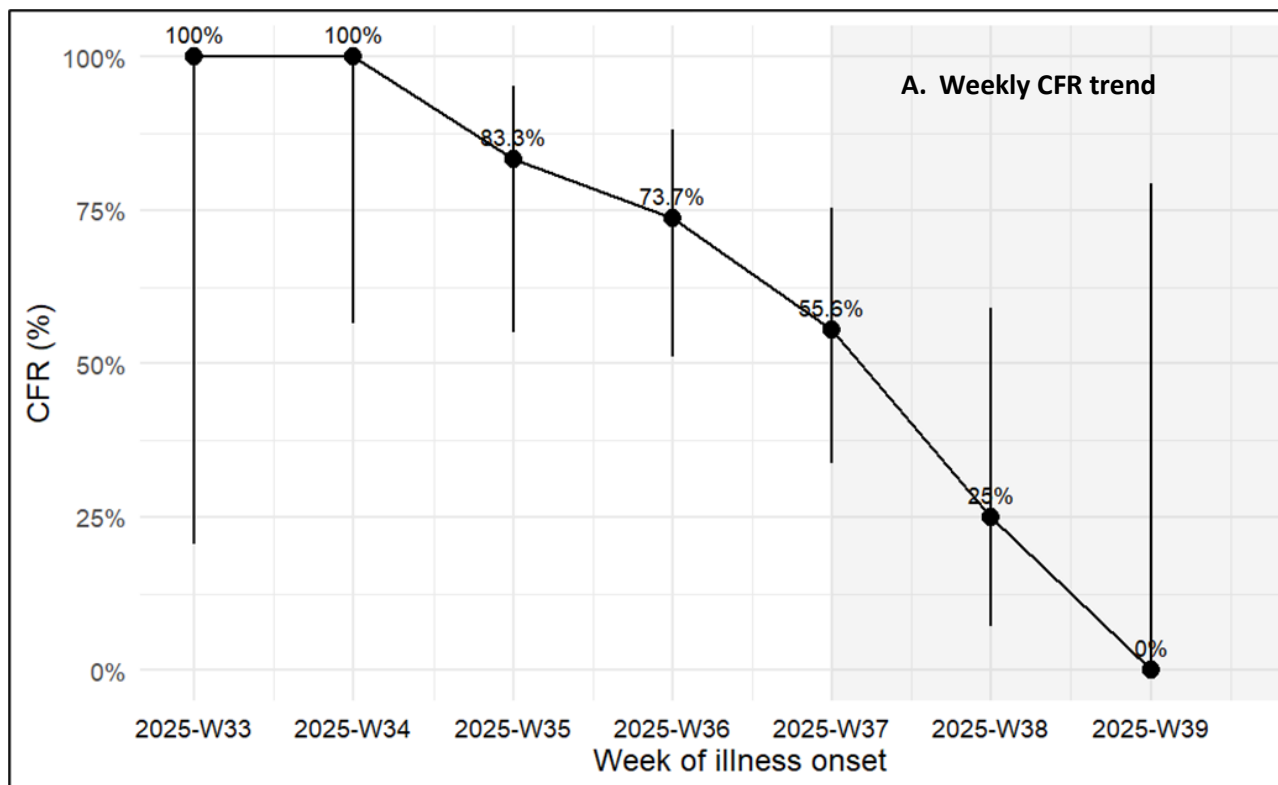


Figure 2: Case fatality ratio (CFR) by week and sex, Ebola virus disease, Kasai province, Democratic Republic of the Congo, as of 28 September 2025



The gray shaded area indicates uncertainty in CFR as some cases in this period are still in treatment unit

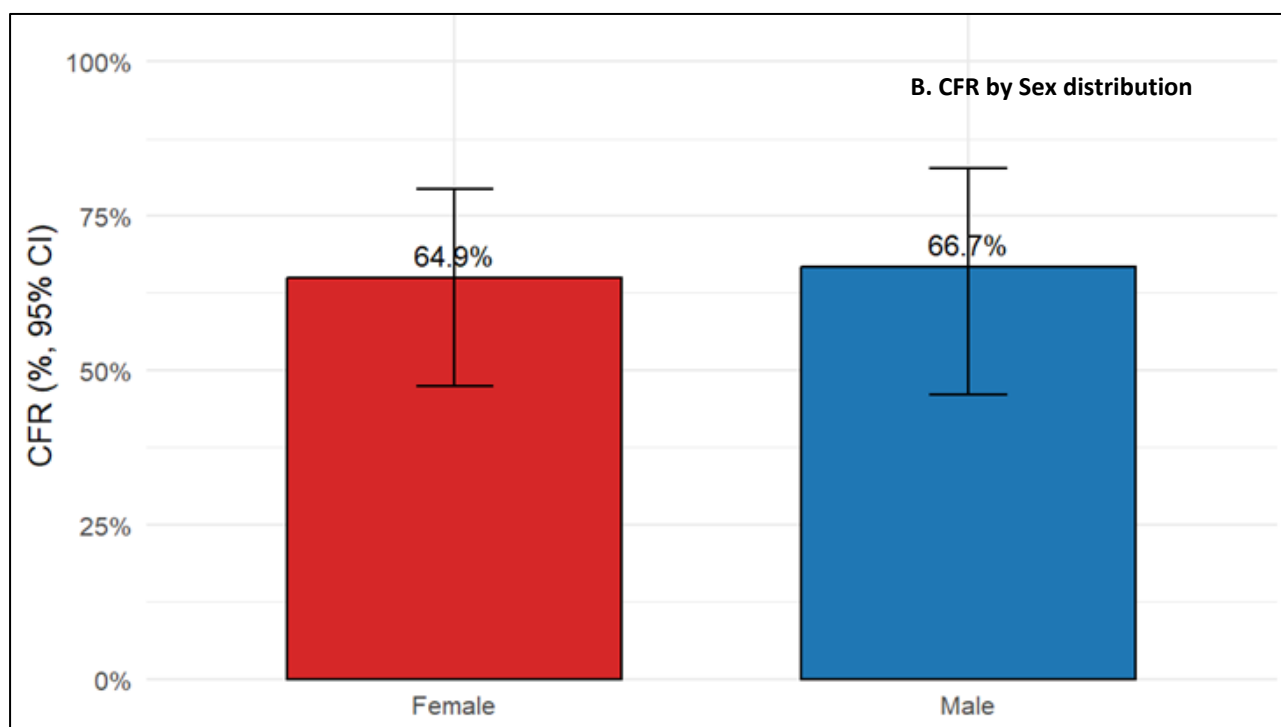
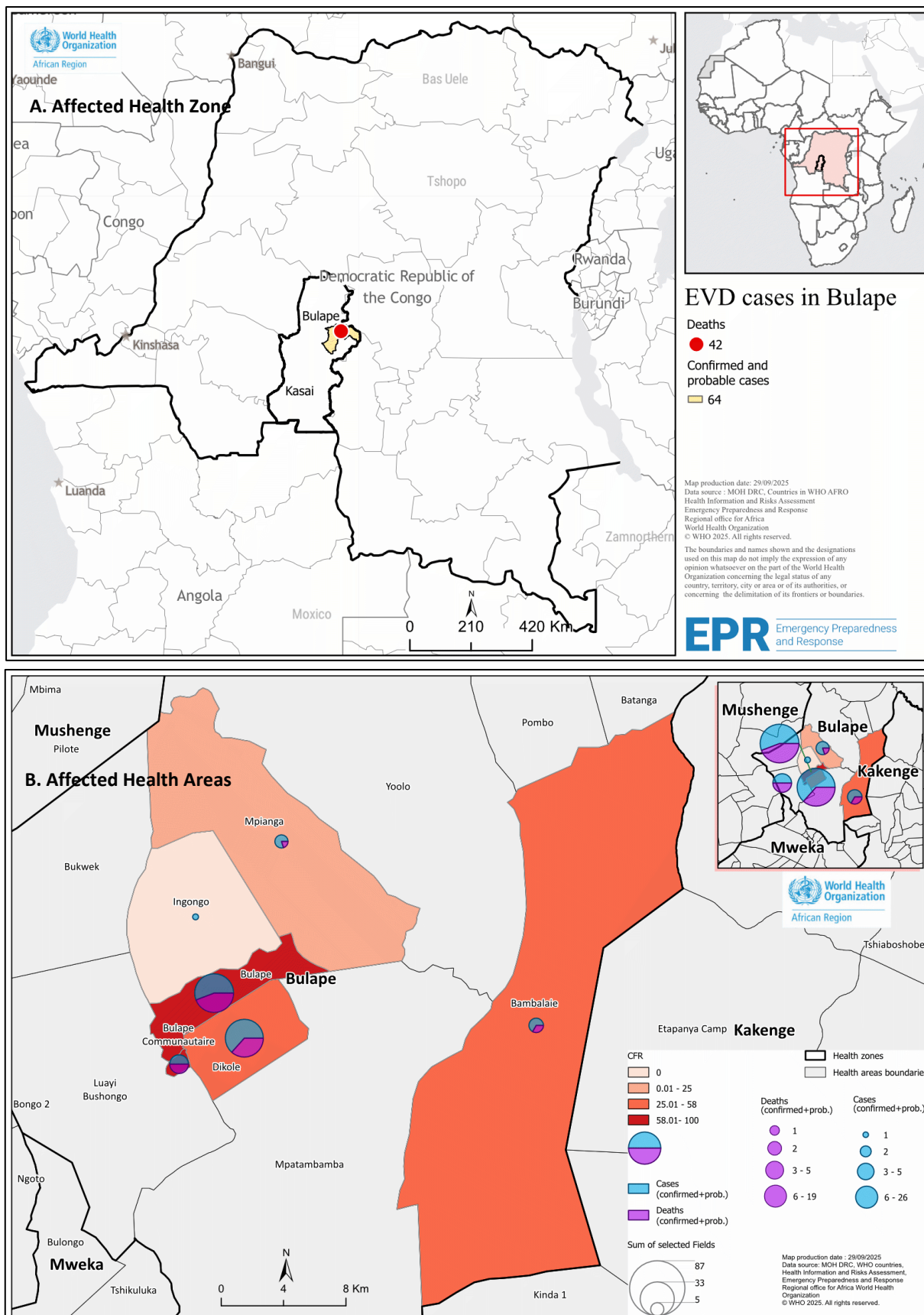


Figure 3: Geographic distribution of cases and deaths of Ebola virus disease, Kasai province, Democratic Republic of the Congo, as of 28 September 2025



2. Public Health Response

Coordination

- The National Incident Management System of the Democratic Republic of the Congo remains activated at the National Public Health Emergency Operations Centre (COUSP) to coordinate response to the outbreak at both strategic and operational levels. The Ministry of Health, through the National Public Health Institute (INSP), continues to coordinate the overall response to the outbreak, with technical support from WHO and health partners.
- At the operational level in Bulape Health Zone, field coordination structures through an integrated Incident Management Team (IMT) remain fully active, ensuring that all aspects of the response are aligned and managed efficiently. Under the leadership of the Ministry of Health, with WHO support, the IMT continues to guide decision-making, prioritize resources and ensure timely communication across response pillars.
- Over the last week, seven (7) additional experts were deployed, giving a total of 77 experts deployed by WHO as of 28 September 2025. Of these 74% (n=57) are nationals, including 17 from the Africa Volunteer Health Corps Strengthening and Utilizing Response Groups for Emergencies (AVOHC SURGE). Of the total experts deployed, 64 are stationed in Bulape Health Zone, epicentre of the outbreak.
- Other health partners supporting response efforts in Bulape include Africa CDC, International Organization for Migration (IOM), UNICEF, MSF, the Alliance for International Medical Action (ALIMA), the International Medical Corps (IMC), the Congolese Red Cross, and US Centers for Disease Control and Prevention. Others providing support include Family Health International 360 (FHI360), Program for Appropriate Technology in Health (PATH), Village Reach, World Food Program (WFP), UNFPA, the GAVI Alliance, Research Triangle Institute (RTI), FCDO, the UK Foreign, Commonwealth & Development Office, DG ECHO and the World Bank.
- A regional strategic preparedness and response plan has been developed to guide coordinated efforts across affected and at-risk areas, focusing on surveillance, diagnostics, vaccination, infection, prevention and control (IPC), community engagement, and operational readiness in neighboring countries.
- WHO has launched an appeal of US\$ 21 million to support the Democratic Republic of the Congo to scale up response operations to interrupt all transmission chains, control the outbreak, and mitigate the impact on human health. So far, US\$2 million has been released from WHO's Contingency Fund for Emergencies to finance immediate response activities.

Surveillance and Laboratory

- Enhanced surveillance activities continue in Bulape Health Zone and surrounding areas. Active surveillance for signals of unusual case presentations, including suspected cases, has been intensified, with 183 alerts reported and investigated during epidemiological week 39 (22 – 28 September 2025).
- All detected cases are being systematically investigated to determine the circumstances of exposure and identify potential contacts. A total of 1 787 contacts were under follow-up, with 1 735 (97.1%) seen on 28 September 2025.

- To strengthen community-based surveillance, 10 motorcycles and 200 tablets were deployed for case detected as well as digital contact tracing, and 21 data managers were trained to improve reporting and analysis.
- As of 28 September 2025, a total of 277 samples have been tested. Of these, 53 were confirmed positive by RT-PCR testing. A mobile laboratory, deployed to Bulape Health Zone since 19 September 2025, continues to facilitate the rapid turnaround of sample testing.
- To strengthen diagnostic capacity, HemoCue rapid diagnostic tools – point-of-care (POC) testing devices designed to provide quick, reliable, and quantitative blood test results – were introduced for real-time patient monitoring. Rehabilitation of the Mweka Laboratory is ongoing to expand testing and improve turnaround times.
- Mortality surveillance has been instituted in health facilities and community to rule out any potential Ebola death. Reviews of health facility registers to identify any missed cases have commenced.

Case Management

- As of 28 September 2025, nine (9) cases had recovered and were discharged and 13 are currently in admission undergoing care.
- The total bed capacity at the Bulape Ebola Treatment Centre (ETC) has further increased from 34 to 49. As of 28 September 2025, the bed occupancy rate was 59.1%, with 13 confirmed cases and 16 suspected cases in admission.
- To date, 31 patients have received treatment with the monoclonal antibody (mAb114), also known by its generic name ansuvimab-zykl (Ebanga). Furthermore, mental health and psychosocial support and nutritional care are being provided to patients in the ETC. Nine of these patients have so far recovered.
- A surge team of health care workers has been deployed by the MoH and Institut National de Santé Publique (INSP) to support clinical care, including trained specialists from the pool of rapid responders and the national emergency medical team.
- ALIMA, MSF, IMC and INSP are actively supporting case management activities.
- Additional staff are being deployed to relieve existing staff at Bulape General Hospital who were listed as contacts so as to ensure continuity of routine care to non-Ebola patients.

Vaccination

- Vaccination with rVSVΔG-ZEBOV-GP (Everbo) Ebola vaccine continues in the affected areas. As of 28 September 2025, a total of 4 115 individuals had been vaccinated over a period of 16 days since the commencement of vaccination activities. These include healthcare and frontline workers, contacts, and potential contacts of cases

- In addition to the ring vaccination approach, targeted geographic vaccination started on 27 September 2025, targeting all high-risk populations in the hotspots where cases have been confirmed. These include Bulape, Bulape Communautaire, Dikolo, Bambalae, Ingongo and Mpianga Health Areas.
- An additional 12 vaccination teams were trained on 26 September 2025 making a total of 18 teams. These have been deployed to scale up vaccination using both the ring vaccination and targeted geographic vaccination strategies. Out of these, four teams are conducting ring vaccination while the other 14 teams are conducting the targeted geographic vaccination.
- Of 12 130 doses delivered to Kasai Province, 4 115 doses have been utilized with 8 015 remaining in stock.

Infection prevention and control (IPC)

- To enhance IPC practices, 130 health workers were trained in Bulape and Mweka on standard and transmission-based precautions and safe care practices.
- Continue to support healthcare workers by ensuring the timely provision of essential IPC supplies, including personal protective equipment (PPE). In collaboration with partners developed IPC & WASH improvement operational plan for the assessed priority healthcare facilities
- Decontamination activities were completed in seven priority healthcare facilities and 23 affected households, helping to break chains of transmission. Additionally, five Safe and Dignified Burials (SDBs) were carried out in Bulape, Bambalaie and Mpatambamba, ensuring safety for both families and responders.
- Three Safe and Dignified Burial (SDB) teams with 24 members are actively conducting burials for both confirmed and suspected cases, completing most within 24 hours of notification.
- Eighteen Red Cross members have been trained to take over SDB responsibilities, supported by adequate supplies (including a recent UNICEF delivery) and a three-month forecast.

Prevention of Sexual Exploitation, Abuse and Harassment (PRSEAH)

- Prevention of sexual exploitation, abuse, and harassment (PRSEAH) continues to be mainstreamed in the response activities. Activities include briefing of all responders, community sensitization sessions, and community risk analysis.
- A two-day training was held for 20 mobilizers and team members, followed by awareness sessions that reached more than 1 300 individuals, including women and youth, reinforcing safe reporting mechanisms and community protection.

Risk communication and community engagement (RCCE)

- Strengthening school-based prevention: Local education authorities endorsed Ebola prevention in all schools, issuing official guidance and supporting handwashing and awareness activities among students.

- Mobilizing trusted leaders: Religious leaders, traditional chiefs, and school principals committed to door-to-door awareness, rapid reporting of suspected cases, and reinforcing preventive behaviors in their communities.
- Capacity building: Eighty frontline actors were trained with a focus on risk communication and community engagement, ensuring consistent, clear, and culturally appropriate messaging.
- Youth outreach: Targeted campaigns around busy community hubs, such as markets, were conducted to engage young people in prevention and early reporting of cases.
- Public awareness at transit points: Awareness sessions were held at the Bulape airstrip, reaching nearly 200 travelers with prevention messages.
- Community engagement around vaccination: Awareness efforts supported ring vaccination activities in affected neighborhoods to build trust and encourage participation.

Operations Support and Logistics

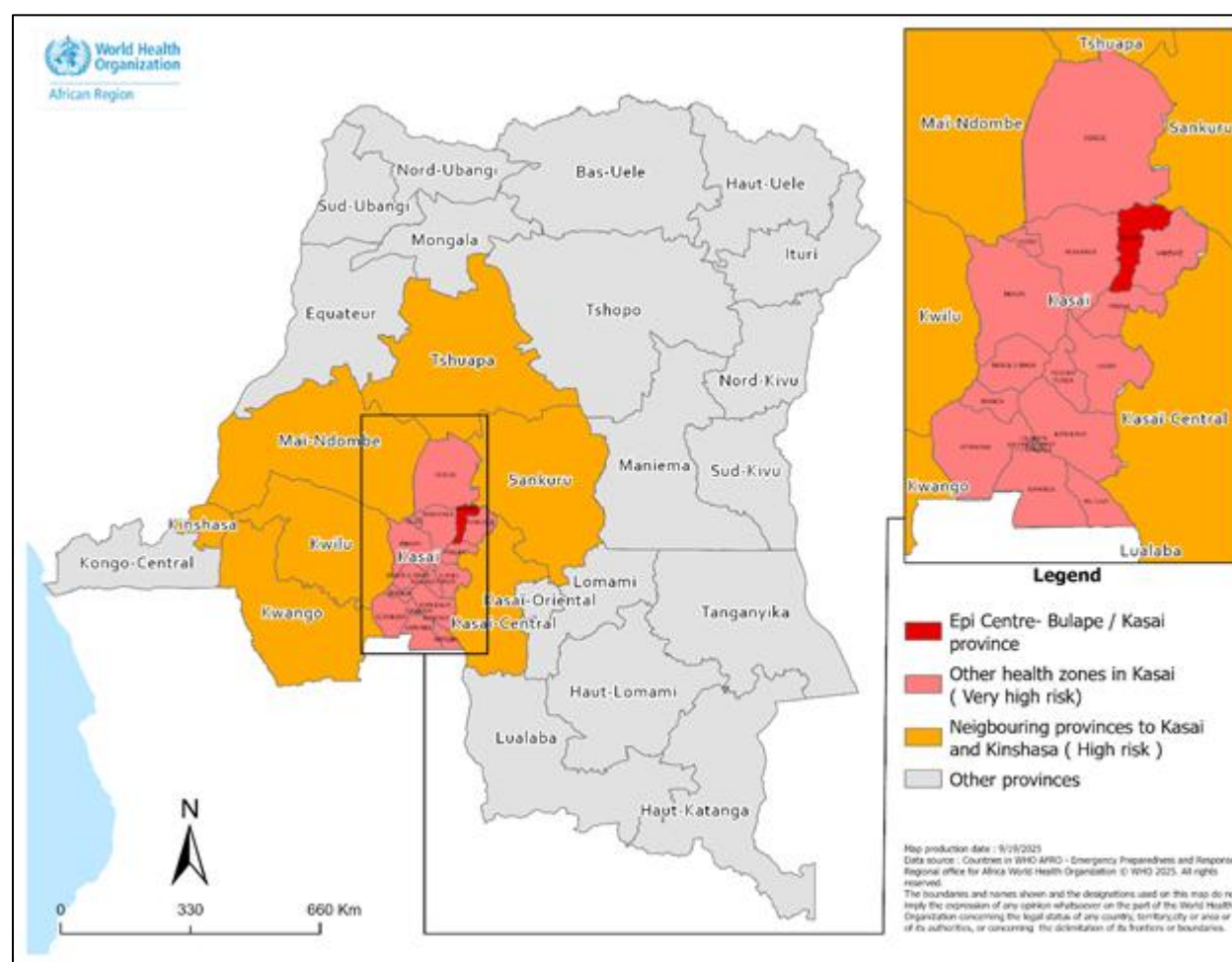
- Logistics operations continued with a focus on expanding access to supplies and infrastructure.
- Essential medical, camp, and construction supplies, including personal protective equipment (PPE), medicines, tents, and vaccine doses were delivered through combined road, and air (helicopter) operations. Local procurement of construction materials further strengthened supply chains.
- Helicopter rotations delivered critical cargo, transported staff, evacuated patients, and supported vaccination campaigns. In total, more than 2.7 metric tons of supplies were delivered on behalf of partners, including WHO, UNICEF, and MSF.
- A 30 km water pipeline was completed, ensuring safe water supply to the ETC, hospital, and staff accommodations. Construction of the ETC screening and triage areas reached 80% completion, alongside progress on latrines, showers, and IPC facilities.
- Field teams installed new generators, solar lighting, and a UPS-powered Starlink internet connection to sustain operations. A generator was also donated to the Ministry of Health to support the Emergency Operations Center.
- Shipments of generators, refrigeration units, PPE, and diagnostic devices were organized from Dakar and Nairobi, reinforcing local response capacity.

Operational Readiness

- All nine neighbouring countries have completed readiness assessments, with an overall regional readiness score of 57%. The results are: Uganda (80%), Rwanda (81%), Burundi (68%), United Republic of Tanzania (65%), Zanzibar (64%), Central African Republic (42%), Republic of Congo (37%), South Sudan (41%), and Zambia (37%). Readiness assessment for Angola has been completed pending analysis.

- Four out of the eight assessed countries have updated or developed their contingency plans (South Sudan, Tanzania, Uganda and Zambia) while others are in advanced stages of development.
- In DRC, the neighbouring high-risk provinces are being supported for readiness assessment and contingency planning. Readiness and preparedness plan for nearby Sankuru Province has been completed. Three other surrounding provinces data have been analyzed and shared for action.
- Readiness training have been planned for five at-risk health zones (Mweka, Mushenge, Luebo, Dekese, Kakenge) on surveillance, IPC, risk communication, and community engagement.

Figure 4: Ebola virus disease risk assessment for health zones and provinces close to the outbreak epicentre as of 28 September 2025, Democratic Republic of the Congo



Domestic and international traffic-related measures and cross-border health

No international traffic-related measures are currently warranted. Health authorities are reinforcing surveillance at border crossings, including health screening, proactive risk communication with travellers at points of entry/points of crossing (PoEs/PoCs), sensitization for PoE staff to detect, report and manage suspected cases, integration of border communities in affected areas into early warning systems and the national surveillance network, as well as coordination with IOM. WHO continues to monitor cross-border risks and provide technical support to mitigate the risk of cross-border spread.

Situation Interpretation

The Ebola outbreak in Kasai Province is showing encouraging signs of decline, with transmission now more localized and less explosive than in the initial phase characterized by nosocomial spread and superspreading events. Although the crude case fatality ratio remains high, improvements in surveillance and expanded access to treatment are contributing to better outcomes among hospitalized patients.

Despite these gains, risks remain. Small clusters within families and close contacts could sustain low-level transmission if not promptly identified and isolated. Surveillance indicators such as alerts, contact tracing, and mortality reviews continue to guide interpretation of transmission dynamics, but the potential for under-detection of community deaths or silent chains of transmission cannot be ruled out. The response therefore must remain vigilant, comprehensive, and sustained until all transmission is interrupted. Continued investment in logistics, surveillance, vaccination, infection prevention and control, and community trust will be decisive in consolidating progress, preventing resurgence, and ultimately bringing the outbreak to an end.

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